ST. JUDE SYNTHETIC HEALTH SYSTEM

Comprehensive Diagnostic & Outpatient Clinical Record

RECORD ID: MED-2026-9901 X
DATE: OCT 22, 2026

PATIENT NAME	DOB / AGE / GENDER	MRN	ATTENDING PHYSICIAN
Evelyn Vance (Synthetic)	04/1 2/1 988 (38y) / F	MRN-8820-91 B	Dr. Aris Thorne, MD

BLOOD PRESSURE	HEART RATE	RESPIRATION	SPO2	BMI
1 1 8/76 mmHg	72 bpm	1 6 /min	99%	22.4 kg/m²

ACTIVE DIAGNOSES & ICD-1 0 CODING

ICD-1 0 Code	Condition Description	Severity	Onset Date	Status
E03.9	Hypothyroidism, unspecified	Mild	02/1 4/2021	Managed
G43.909	Migraine without aura, intractable	Moderate	09/1 0/2023	Review
M54.50	Low back pain, unspecified	Acute	1 0/01 /2026	Improving

CRITICAL ALLERGY ALERT: Patient has a documented severe reaction to *Penicillin G* (Anaphylaxis reported 201 8). Alternative antimicrobial protocols required.

CURRENT PHARMACOTHERAPY & DOSAGES

Levothyroxine Sodium 75 mcg

Directions: Take 1 tablet daily by mouth 30 minutes before breakfast.

Refills Remaining: 4 | **Rx ID:** RX-4401 -A

Sumatriptan Succinate 50 mg

Directions: Take 1 tablet at onset of acute migraine symptoms. Repeat after 2h if needed (Max 1 00mg/24h).

Refills Remaining: 2 | **Rx ID:** RX-91 82-M

I LABORATORY PANEL RESULTS (SERUM METABOLIC)

Assay / Panel	Observed Value	Reference Range	Units	Flag
Thyroid Stimulating Hormone (TSH)	2.45	0.40 - 4.50	mIU/L	Normal
Serum Creatinine	0.88	0.60 - 1.10	mg/dL	Normal
Fasting Plasma Glucose	104	70 - 99	mg/dL	Elevated
Serum Alanine Aminotransferase (ALT)	22	7 - 56	U/L	Normal
Total Serum Cholesterol	188	< 200	mg/dL	Normal

V PHYSICIAN SOAP NARRATIVE NOTES

SUBJECTIVE: Patient returns for 6-month routine follow-up regarding hypothyroidism and migraine management. Reports good compliance with Levothyroxine. Migraine frequency has decreased to ~1 episode per month, responsive to Sumatriptan. Mild lower back strain reported following physical exercise 3 weeks ago.

OBJECTIVE: Vital signs stable and within normal limits. Thyroid gland non-palpable, no tenderness or nodules noted. Neurological screen clear. Lumbar range of motion slightly reduced on flexion, no radicular signs.

ASSESSMENT & PLAN: Continue current Levothyroxine 75mcg dose based on normal TSH. Maintain Sumatriptan PRN. Recommended lifestyle modifications for borderline fasting glucose (104 mg/dL); re-check HbA1c in 6 months. Physical therapy exercises provided for lumbar strain.

ELECTRONICALLY SIGNED BY:
Dr. Aris Thorne, MD (Internal Medicine)
NPI: 9918203911 | DEA: BT-9918273

ATTENTION: THIS IS A SYNTHETIC TEST RECORD ONLY. IT IS NOT A REAL ELECTRONIC HEALTH RECORD (EHR) AND HAS NO MEDICAL OR LEGAL VALIDITY.